

Patient & Prescriber Information Leaflet

Allopurinol Tablets

What Allopurinol is used for

Allopurinol is a xanthine oxidase inhibitor used for the long-term management of gout and hyperuricemia. It is indicated for patients with recurrent gout attacks, gouty tophi, or elevated serum uric acid levels. Typical presenting symptoms include sudden severe pain, swelling, redness and warmth in a joint (classically the big toe), often recurring.

Relevant laboratory markers

Serum uric acid greater than 6.8 mg/dL (or greater than 7.0 mg/dL in men) supports a diagnosis of hyperuricemia. During an acute flare, CRP and ESR may also be mildly elevated due to joint inflammation.

Dosage and administration

The usual starting dose is 100 mg once daily, titrated upward every few weeks based on serum uric acid levels, to a typical maintenance dose of 100-300 mg once daily (maximum 800 mg/day under specialist supervision). Allopurinol should not be started during an acute gout attack; it is used for long-term prevention once the acute flare has settled with anti-inflammatory treatment (e.g. NSAIDs or colchicine).

Contraindications

Known hypersensitivity to allopurinol, prior severe cutaneous adverse reaction to allopurinol (e.g. Stevens-Johnson syndrome), and concurrent use with azathioprine/mercaptopurine without dose adjustment (allopurinol markedly increases their levels).

Side effects

Common: rash, gastrointestinal upset. Rare but serious: severe hypersensitivity syndrome, Stevens-Johnson syndrome / toxic epidermal necrolysis, hepatotoxicity. Patients of certain genetic backgrounds (HLA-B*5801 positive) are at higher risk of severe skin reactions.

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